

History and Concerns
(Share key information gathered from team)

Elizabeth, a 4-year-old girl, was referred to her local cleft lip and palate clinic for concerns with hypernasal speech, poor intelligibility, and dysmorphic features. Elizabeth has been enrolled in speech-language therapy for 6 months for articulation. Her current speech-language pathologist (SLP) identified concerns with hypernasal speech and made a referral for further interprofessional (IP) team evaluation. Elizabeth's parents agreed with the referral for an IP evaluation.

Assessment Results
(Summarize key diagnostic results)

The team meets to share the evaluation results and to develop a coordinated treatment plan:
SLP – The cleft palate team SLP reports that Elizabeth's speech is characterized by glottal stops, glottal coarticulation, nasal substitutions, and final consonant deletions. Parents report typical play and social skills. The SLP judges Elizabeth's speech intelligibility as significantly below age expectations, and parents report that they have a difficult time understanding Elizabeth.

IPP Treatment Plan
(Discuss, reflect, and modify recommendations to develop a coordinated plan)

Based on the reported findings, the IPP team develops and agrees on a treatment plan, which the team facilitator will communicate to the family. The plan includes the following actions:
SLP – Continue Elizabeth's current speech-language therapy, targeting articulation. The team recommends a full language evaluation.

Assessment Plan
(Determine roles/responsibilities for evaluation)

The interprofessional cleft lip and palate team members include a SLP (team facilitator), nurse practitioner (NP), audiologist, plastic surgeon, psychologist, geneticist, social worker, and ENT. Team members meet to discuss Elizabeth's assessment needs and together agree upon areas of assessment as well as team roles and responsibilities. The team determines that all members will evaluate Elizabeth in the same clinic, as part of a team visit, and then meet as a group to discuss observations and recommendations. The assessment plan for the team visit is as follows:
SLP – Conduct (a) perceptual speech evaluation with a focus on evaluation of resonance and any obligatory features of velopharyngeal disorder (VPD); (b) standardized articulation assessment; (c) oral examination; and (d) instrumental assessment, if indicated.
NP – Conduct case history as well as developmental and general medical assessment.
Audiologist – Evaluate hearing; assess middle ear status.

Treatment Outcomes
(Discuss results of treatment)

At cleft palate clinic the following month, the results from the microarray confirm the 22q11.2DS diagnosis. Elizabeth and her parents return to the cleft palate clinic for a nasopharyngoscopy and to discuss continuation of the treatment plan:
SLP – Speech therapy is ongoing, targeting the sounds /p/, /b/, /t/, /d/, /k/, and /g/. Treatment incorporates nasal occlusion in therapy and in home practice to provide the sensation of oral airflow and pressure. The SLP maintains frequent contact with the team.